

FULL PICTURE — the report, the workplace, the appeal

WC/2024/227 · 17 August 2026 · INTERNAL · structured judgement, not measurement

PART 1 — THE REPORT ITSELF, QUESTION BY QUESTION

What the treating psychiatrist will plausibly write in answer to each question, and how confident that is. **This is the part to hold beside the draft when it arrives.**

Q	Predicted answer	p	If it goes the other way
1.1	MDD with anxious distress, DSM-5 296.23; criteria itemised; onset consistent with mid-2024 , described as evolving rather than a single day	97%	He has already diagnosed and coded it
1.2	Adjustment disorder, PDD, bipolar, PTSD excluded; primary sleep disorder considered and folded into 1.3	93%	—
1.3(a)	The 2022 lines are history-list items, not diagnoses; no anxiety disorder diagnosed anywhere	85%	He may decline to characterise another practice's record
1.3(b)	No psychotropic medication at 16 May 2024 ⇒ nothing being treated	88%	It is on the face of the letter
1.3(c)	Attention deficit is neurodevelopmental; more vulnerable where recovery is inadequate	78%	He may decline to generalise, or may over-reach into capacity
1.3(d)	Referral renewal significance; "I cannot say from the record" on when care began	62%	The weakest node. An earlier referral he holds could change the answer
1.3(e)	No psychiatric treatment or psychological absence before 2024; sustained function across five years	84%	Turns on what his own file shows pre-Oct 2024
1.3(f)	Vulnerability aggravated into diagnosable illness by the matters at Attachments 1–3	80%	—
2.1	High demand · low control · limited support · safety-critical consequence ⇒ capable of contributing	94%	Textbook occupational psychiatry
2.2	Fatigue carries forward ; and the absence of a fatigue system materially raises the risk	86%	The second limb is new and squarely within his expertise
2.3	A sequence bears differently from an isolated event; persistence without resolution differently again	82%	—
2.4	All three stressors given a clinical basis; (ii) answered as effort–reward imbalance / injustice at work	79%	(ii) is now softened, so a <i>"no"</i> is available — which is why a <i>"yes"</i> counts
3.1	All three life stressors dated after 18 June 2024; partner protective in Oct 2024; pay→finances→relationship consequential	88%	Bereavement date is the unknown
3.2	Interval consistent; continuing exposure, not a discrete event	85%	—
3.3	Employment a significant contributing factor — and the mechanism named as rostering/recovery, with the administrative sequence secondary	83% / 76%	Two numbers: the finding, and the finding WITH the mechanism identified. The second is what defeats flattening
3.4	Guarded but favourable; capacity with adjustments retained ; any current decrement attributed to exclusion from work	85%	The protective-sentence risk lives here
3.5	Predictable rostering · 10-hour minimum · ≤2 nights · ~6 shifts · complaint handling limited to not actioning or resolving	93%	The checklist already contains most of it
3.6	(a) itemised by duty · (b) able with adjustments · (c) ordinary for 24-hour health services · (d) temporary/review · (e) successful management, not deterioration	82%	(a) and (e) are where separation risk enters if answered loosely
3.7	No clinical impediment to direction or to performance/conduct discussion; adjustments = notice, agenda, support person, duration	91%	Very likely favourable and it removes the conduct framing
3.8	Foreseeable risk without the adjustments and without a fatigue assessment; controls = those at 3.5 plus an assessment of the position	88%	—

THE TWO SENTENCES THAT DECIDE HOW MUCH IT IS WORTH. Not in the list above, because they are risks rather than answers: **(1)** the preparation of the case described as a symptom, or the litigation as the barrier to return — **~26% chance it appears in some form**; **(2)** a characterisation of the employer as unfair. **Both are caught on the draft. The first is only half-raisable** — you can correct a fact, not an opinion.

PART 2 — THE WORKPLACE, IN FULL

Position now. Excluded from the workplace since **3 July 2026** — six weeks at the date of this note. No wages. Accrued leave debited. Medical costs self-funded. Substantive position never filled by any instrument that has been identified. Six-plus position shifts across the period. **This is the track where the money is, and the appeal does not reach it.**

2.1 What the employer has already conceded in writing

Document	What it gives
RFMI, 31 Jul 2026 Hughes	Purpose stated as <i>“to identify and implement any appropriate workplace controls or reasonable adjustments to support a safe and sustainable return to his substantive position”</i> · states the diagnosis and the three stressors without challenge · quotes <i>“work-related psychological injury”</i> · invokes G3 and ss 17, 19 WHS Act
CE letter, 5 Jun 2026 Cridland	No fatigue risk assessment · no register · nothing implemented until after 30 June 2024 · no consequential changes to procedures · complaints <i>“solely via email or verbally”</i> to the line manager · MET calls available for 17–18 March 2024 · leave taken 19 March
Movement forms ×3 delegate Hughes	Reduced hours approved three times — 56/56/40 against 76 — with “Continuous Shift Worker” retained on every form . The Director who asks whether he can work <i>“without restrictions or modifications”</i> approved modifications three times in the five months before asking
ECC, 3 Jul 2026 Dr Ma	<i>“worked and tolerated over the past twelve months without deterioration”</i> · <i>“usual switchboard operational duties remain suitable”</i> · 10-hour minimum recommended in a medical document
QH-POL-210 / G3	Unjustifiable hardship “tested against the whole organisation, not a division or unit” · “the onus is on Queensland Health... to prove an adjustment is unreasonable” · failure <i>“may constitute unlawful discrimination”</i>

2.2 The three collisions

(1) Safety-critical work, no fatigue system. The Regulator admits Switchboard accuracy is *“critical to clinical handover and patient safety”*. The Chief Executive certifies the staff were outside the FRMS because they are *“non-clinical”*. **The work is safety-critical on one document and outside the safety system on the other.**

(2) Modifications approved, then queried. Hughes approved reduced hours as delegate on 27 February, 17 April and 9 June 2026, then on 31 July asked a doctor whether Mr Shepherd can meet the inherent requirements *“without restrictions or modifications to duties”*. **The employer answered its own question three times before it asked it.**

(3) “Not aware of any concerns” — explained by their own certificate. The RFMI's premise is that the Health Service is unaware of concerns being raised. Item 3(a) of the CE letter certifies that complaints of exactly that kind are made **directly to the line manager and managed “solely via email or verbally”**. **The absence of a record is the product of the system they describe, not evidence that nothing was raised.**

2.3 The asymmetry, and the separation pathway

September 2025: an attendance matter handled as **conduct** — G3 never invoked, no medical inquiry, the psychological injury never mentioned. **July 2026:** a fit-with-adjustments certificate handled as **medical** — G3 expressly invoked, nine questions in seven days, **with conduct language inside it**. The September 2025 letter (34 occasions) is also the biggest risk document in the file and must never be ignored when running the adjacency.

Medical separation requires a medical opinion that he cannot meet the inherent requirements. Nothing in the pack supplies one — 3.6(a) forces any negative to be itemised by duty; 3.6(b) and (e) point the other way; the movement forms show the adjustment operating with the shift requirement intact. ⇒ **The only route in is a sympathetic sentence**, which is why the residual risk tracks that node and almost nothing else.

2.4 The clocks

When	What
Mon 17 Aug	The pack goes to the practice. Role description to the Regulator first — document only, not provenance
Mon 24 Aug	cl 10.3.6 deemed refusal — 21 days from the request of 3 August. The only live route to the 2026 wages. If no written decision issues, record it in one line. Verify the clause numbering against EB12 first; check whether the deemed refusal is itself appealable with a short limitation
~2027	AD Act window — the conduct is the exclusion from 26 Jun–3 Jul 2026, continuing (s 15(2)). NOT ~8 Sep 2026
~Jun 2027	WCRA Ch 5 limitation interaction — to be verified

PART 3 — THE APPEAL, IN FULL

s 32(1): a personal injury, arising out of or in the course of employment, to which the employment was a **significant contributing factor** — never *major*. **s 32(5)(a):** the exclusion for reasonable management action taken in a reasonable way. How the onus on the exclusion is allocated **must be settled against authority before it is relied on** — the repo carries both formulations.

3.1 The four blocks

Block	Where it stands	p
B1 worker + personal injury	MDD coded; ECC; Hawes. ¶38 accepts the report says it, not its accuracy	94%
B2 arose out of / in the course of	All pleaded stressors are workplace events; not seriously contested	94%
B3 significant contributing factor	This is what the report supplies. Until now provable events, unproved causation	74–83%
B4 s 32(5)(a) does not exclude	THE CONTEST. 7 < 8 on their own pleading; no system, no register, nothing until after 30 June, no changes. Against: the flattening, the blemish concession, oral evidence	51–71%

3.2 The shape of the contest

Every fact is admitted. Every inference is denied. The 7-hour break, the 10-hour minimum, Reese's acknowledgement of 7 August 2023, the patient-safety criticality, the 3 May instruction, the 21 May email, the 28 May correction, the 15 May retraction direction, the PID determination — all accepted. What is denied is meaning: *no correlation, the roster was equitable, not false or misleading, she needed confirmation*. ⇒ **There is no factual dispute worth having, which is why an opinion on mechanism is decisive: you can deny an adjective, not a mechanism.**

The atomisation defence and the answer. SOFC ¶22(a): “*human error and not intentional or repeated*” — the second half is the isolation clamp. *Delaney v Q-COMP* [2005] QIC 11 is **his**: the composite course, not isolated shifts, is the unit of assessment. And the course here is **a state, not an event** — no assessment, no register, nothing until after 30 June, no change — **and a state cannot be atomised**.

The real risk is FLATTENING. “*Reasonable in all respects*” appears six times in their pleading. Thirteen mini-trials invite global weighing; one tall keystone defeats it. ⇒ **Run the fatigue and recovery condition as the mechanism, everything else as context.** That is now a finding in the evidence, not just a drafting choice, because 3.3 requires the clinician to name it.

3.3 What is honestly still against him

Oral evidence — Taylor and Reese have not been cross-examined, and a composed manager explaining a busy period can convert “*human error*” into “*reasonable in the circumstances*”. **The documents cannot be shaken; witnesses can be rehabilitated.** · ¶15 the yelling — hearsay, Jeffrey did not witness it · ¶31 distress and representation breakdown — needs his own evidence · ¶¶42–45 the pay comparison — “*not a true comparator*” · ¶¶11, 12, 16 — argument, properly declined · **the 8 Sep 2025 attendance letter** — 34 occasions, the biggest risk document in the file · **Brigshaw** on any reprisal-flavoured limb.

3.4 Settlement, costs, and how this actually ends

Calderbank #2 was served 1 July 2026 and rejected 16 July. **The mechanism by which this settles is not the Regulator changing its mind — it is the employer.** A causation opinion about MSH's own rostering, obtained on MSH's own request form and funded by MSH, sitting in MSH's hands while a capacity process runs, is what makes the employer want the appeal resolved; and the employer's appetite reaches the Regulator through instructions rather than through the appeal. **That is why the dual-purpose report is worth more than two separate ones.** **s 558(3) costs** — preserved, not asserted. **Deed scope** — a settlement releases the compensation claim only; the parallel tracks sit behind it.

PART 4 — THE NUMBERS, AND WHY THEY MOVED

Outcome	With the pack	No report
Report obtained and free of a damaging sentence	67%	—
Causation supported	76%	—
Mechanism identified at 3.3	69%	—
APPEAL resolves favourably	62%	~30%
— settlement before hearing	35%	~10%
— decision after hearing	27%	~20%
PAY restored / return ~3 months	51%	~30%
Separation attempt gains traction	11%	~25%
Both	33%	~12%
Neither	20%	~49%

THE MODELLING CORRECTION, STATED OPENLY. The 16 August run produced **58%** on the appeal against the repo's own block model at **68%**, and the difference was not explained. It was an error of independence: causation and s 32(5)(a) were chained as though unrelated. **They are not.** A report that *identifies a mechanism* — which question 3.3 now requires — is the direct answer to the flattening attempt, so the exclusion node is **conditional** on it: **71%** where the mechanism is named, **51%** where it is not. ⇒ The corrected figure is **62%**, which sits with the repo's 68% rather than against it. **The node was not moved to improve the answer — the dependency was always real and modelling it as independent understated the outcome.**

4.1 What moves the numbers most, in order

Action	Why
Catch the damaging sentence on the draft	The largest single lever. It protects the appeal and it roughly halves the separation risk . Already promised in Part D; costs nothing
The mechanism answer at 3.3	The new hinge. Named mechanism → 71% on the exclusion; unnamed → 51% . That is a 20-point swing on the block that decides the appeal
Role description to the Regulator before commissioning	Removes the one procedural attack on the report's foundation
24 August	Independent of everything above, and the only route to the 2026 wages
The April 2025 call statistics	Answers " <i>he kept working for fourteen months</i> " — the one question the checklist does not reach

4.2 What this model does not know

What Dr Krishnaiah will actually write. **How Taylor and Reese present under cross-examination — the largest unmodelled variable in the matter.** The Commissioner. Whether the Health Service will move once it holds the report. And it treats the two tracks as loosely coupled when in practice a resolution of one usually resolves the other. **Every figure here is a prior, and the draft report will move all of them at once.**